NEUROMETRIC COMPREHENSIVE ASSESSMENT DOSSIER

Full Multi-Battery Cognitive Architecture, GPT Clinical Audit & Psychiatric Screening Profile

Candidate Evaluation: Verified Assessment Profile	**Audit Framework:** CHC v3.1 / Wechsler Norms / GPT Synthesis
Normative Scale: N(100, 15) Standard Population	**Verification Status:** Certified Complete Across All Batteries
Measurement Model: Item Response Theory (IRT)	**Telemetry Timestamp:** 2026-10-21 UTC

1. Primary Cognitive Intelligence Battery: Cattell-Horn-Carroll (CHC) Model

Full-Scale IQ (FSIQ)	Percentile Rank	Classification	95% Conf. Interval
143	**99.7th Percentile**	**Very Superior (Gifted)**	**135 - 147** SEM: 3.1 pts

CHC Cognitive Factor	Cognitive Subtest Construct	Raw Performance	Standard Score	Percentile
Gf (Fluid Reasoning)	Raven's Progressive Matrices (Induction & XOR Logic)	8 / 8 Correct (100%)	**135**	99.0th
Gwm (Working Memory)	Visuospatial Corsi Block Span (Reverse Recall)	8 Blocks Max Span	**138**	99.4th
Gq (Quantitative Induction)	Higher-Order Mathematical Sequences & Algorithms	6 / 6 Correct (100%)	**135**	99.0th
Gs (Processing Speed)	Rapid Symbolic Array Discrimination (45s Burst)	25 Correct, 1 Error	**128**	96.9th

Psychometric Profile Structure: Standard scores range from 128 (Gs) to 138 (Gwm). The inter-factor discrepancy is 10 standard points, which falls well below the 23-point divergence threshold (WAIS-IV / Lichtenberger & Kaufman). The FSIQ score of 143 represents a unitary, highly reliable composite of general intellectual g-factor reasoning.

Comparative Profile: Historical Audit Case vs. Current Audited Assessment

Metric / Construct	Historical Baseline (GPT Audited)	Current Certified Run	Clinical Interpretation & Delta
Full-Scale IQ (FSIQ)	112 (High Average, 78.8th %ile)	**143 (Very Superior, 99.7th %ile)**	Shift from incomplete baseline to validated run
Fluid Reasoning (Gf)	123 (Superior, 93.8th %ile)	**135 (Very Superior, 99.0th %ile)**	Flawless pattern induction & matrix XOR logic
Working Memory (Gwm)	114 (High Average, 82.5th %ile)	**138 (Very Superior, 99.4th %ile)**	Extended spatial buffer retention beyond 5 units
Quantitative (Gq)	109 (Average, 72.6th %ile)	**135 (Very Superior, 99.0th %ile)**	Mastery of non-linear sequences & series logic
Processing Speed (Gs)	78 (Borderline Low, 7.1st %ile)	**128 (Superior, 96.9th %ile)**	Resolution of device latency & key input friction
Discrepancy Status	**45 Points** (Non-Unitary: Gf vs Gs)	**10 Points** (Unitary: Normal Variance)	GAI required in baseline; FSIQ valid in current run

2. Deep Integration of the ChatGPT Psychometric Audit

Connecting every finding and recommendation from the prior clinical audit directly to psychometric and neurological principles.

GPT Audit Finding	Original Quiz Flaw (As Flagged by GPT)	NeuroMetric Engine Scientific Fix
1. Incomplete Assessment	Page 1 showed 75% completion while processing speed was still actively running.	Strict state-machine blocking; scores are never computed until all subtests reach 100% certified completion.
2. Missing Telemetry	Mean item latency displayed as '--' on the report despite computing speed scores.	High-resolution DOM event timers record exact millisecond reaction times for every individual item.
3. Conflicting Percentiles	Report stated 78.8th percentile but claimed 'higher than 97 out of 100 individuals.'	Gaussian cumulative distribution function F(z) mathematically enforces exact percentile mapping (e.g., 78.8% = 79/100).
4. Contradictory Narrative	A score of 78 (borderline low) was euphemistically labeled as 'balanced processing speed.'	Replaced contradictory labels with clinical discrepancy analysis. Low Gs is diagnosed as a speed-accuracy trade-off.
5. Unverified Normative Claims	Report claimed N=10,000 IRT validation without disclosing SEM, confidence bounds, or formulas.	Fully documented CHC model with SEM (3.1 - 3.67), 95% Confidence Intervals, and explicit psychometric disclaimers.

The GAI vs. FSIQ Clinical Principle: When Does FSIQ Lose Validity?

As ChatGPT highlighted in its clinical review, **the most significant finding was the 45-point gap** between fluid reasoning (123) and processing speed (78). Under clinical assessment guidelines (Wechsler WAIS-IV / Lichtenberger & Kaufman, 2009):

• **When discrepancy exceeds 1.5 standard deviations (23 points)** between core factors, the Full Scale IQ is deemed *statistically non-unitary*. FSIQ averages together fundamentally divergent neurocognitive systems, obscuring true intellectual ability.

• In the historical baseline, calculating the **General Ability Index (GAI)** by isolating Gf and Gq yields a true cognitive potential of **120–121 (Superior Range)**.

• **Why Low Processing Speed (78) Does NOT Mean Low Intelligence:** A lower Gs score often reflects deliberate error-checking, sensory-motor interface friction (mouse/keyboard input delay), or fatigue. It represents throughput bandwidth, not conceptual or reasoning competence.

The Tri-Pillar Model: Translating Psychometrics into Real-World Ambitions

Cognitive Pillar	Psychological Construct	Operational Meaning for AI, Tech & Security
1. Reasoning (G_f, G_q)	Pattern induction, novel problem solving, matrix XOR logic	Exceptional ability to design algorithmic architectures, reverse-engineer complex systems, and connect cross-domain concepts without prior templates.
2. Retention Buffer (G_{wm})	Working memory span, multi-variable state tracking	Performance remains high up to 5-6 mental variables. For complex systems, externalizing state (written notes, modular architecture) prevents cognitive overload.
3. Throughput & Speed (G_s)	Visual scanning, key latency, rapid discrimination	Prioritizes precision and deep verification over impulsive reflex. In production engineering, this manifests as rigorous code auditing and fewer critical regressions.

3. Advanced Memory Recognition: Delayed Match-to-Sample (DMTS)

Total Completed Trials	Recognition Accuracy	Visual Retention Index	Mean Response Latency
40 Trials	**95.0%** (38/40)	**139** (99.5th %ile)	**1.24s**

Visual Working Memory Consolidation: Tested under the PEBL research laboratory protocol (2.5s encoding exposure, 1.2s mask delay). 95.0% accuracy across 40 distinct trials confirms that short-term visuospatial buffer consolidation is fully intact when perceptual interference is controlled.

4. Executive Neuroplasticity Tracker: Dual N-Back Working Memory

Continuous Steps	Updating Accuracy	Plasticity Reserve Index	Clinical Reserve Band
40 Steps (N=2)	**92.1%** (35/38 hits)	**138** (99.4th %ile)	**High Cognitive Reserve**

Dorsolateral Prefrontal Plasticity: Scoring 92.1% on a 40-step continuous Dual N-back battery proves an extraordinary ability to update working memory contents dynamically while inhibiting proactive interference from past trials.

5. Dark Triad & Multiple Intelligences Architecture

Psychometric Dimension	Evaluated Construct	Affinity %	Clinical Classification
Strategic Calculation	Machiavellian tactical planning & utility optimization	**90.0%**	High Strategic Foresight
Crisis Composure	Stress immunity, low neuroticism, emotional stability	**97.5%**	Exceptional Acute Crisis Composure
Logical-Mathematical	Algorithmic induction, formal proofs, probability	**100.0%**	**Dominant Peak**
Spatial-Visual	3D topological rotation, geometric mechanism design	**100.0%**	**Dominant Peak**

6. Comprehensive Problem Taxonomy: Constructs & Tasks Solved

Item-by-item structural documentation of the mathematical, logical, and neurocognitive mechanisms successfully solved across the full assessment session.

Assessment Battery	Problem Construct Solved	Specific Mechanisms & Computational Logic	Accuracy
Fluid Matrix (Gf) 8 Items	Topological & Boolean Matrix Induction	• **Rotational Symmetries:** Solved 90°/180° clockwise angular progressions. • **XOR Logic Overlays:** Resolved overlapping geometric grids where intersecting lines cancel out. • **Concentric Scaling:** Inferred dual-rule scaling factors across nested polygons.	**100.0%** (8/8)
Spatial Memory (Gwm) 16-Block Matrix	Visuospatial Sequence Span Retention	• **Inverted Recall:** Reconstructed reverse path sequences across 16 matrix coordinates. • **Span Length 8:** Maintained high-load spatial coordinate vectors without boundary errors. • **Delay Resistance:** Retained target nodes across 1000ms inter-stimulus flash intervals.	**100.0%** (Span 8)
Quantitative (Gq) 6 Series	Higher-Order Mathematical Deductions	• **Quadratic Delta:** Solved second-order derivative sequence: 2, 6, 12, 20, 30 → 42. • **Dual-Predecessor Sum:** Fibonacci-type series synthesis: 3, 5, 8, 13, 21 → 34. • **Multi-Operator Alternating:** (+3, ×2, +3, ×2): 2, 5, 10, 13, 26 → 29.	**100.0%** (6/6)
Processing Speed (Gs) 45s Rapid Burst	High-Throughput Visual Discrimination	• **Array Scanning:** Evaluated 6-symbol arrays for target glyph in <1.2s per trial. • **Distractor Invariance:** Correctly rejected high-similarity perceptual lures. • **Precision Calibration:** Maintained 96%+ accuracy under high-speed countdown constraints.	**96.2%** (25/26)
Memory DMTS 40 Vector Trials	Delayed Visual Matching to Sample	• **2.5s Target Encoding:** Consolidated complex polychromatic geometric targets. • **1.2s Mask Buffer:** Prevented iconic trace decay during visual interference. • **4-Choice Discrimination:** Isolated true target from 3 rotated and scaled foils.	**95.0%** (38/40)
Dual N-Back 40 Continuous Steps	Executive Working Memory Updating	• **Dual Modality Tracking:** Co-monitored spatial grid positions and audio letter tokens. • **Interference Filtering:** Inhibited N-1 and N-3 perceptual lures to isolate N=2 matches. • **Cognitive Plasticity:** Maintained dorsolateral prefrontal updating without performance drift.	**92.1%** (35/38)

7. Clinical Neurodivergence & Psychiatric Screening Battery

Comprehensive screening across 8 standardized DSM-5 & WHO clinical instruments evaluating attention, perception, obsessive-compulsive traits, and executive function.

Clinical Screener	Standardized Model	Evaluated Construct	Cutoff	Risk Tier
ADHD Screener	WHO ASRS v1.1 (18 items)	Inattention, task initiation, motor restlessness & working memory lapses	≥ 14 pts	**Minimal Risk**
Schizotypy	SPQ-B (22 items)	Cognitive-perceptual oddities, magical ideation & disorganized thought	≥ 12 pts	**Sub-clinical**
OCD Spectrum	OCI-R (18 items)	Checking, washing, ordering, obsessing, neutralizing & hoarding distress	≥ 21 pts	**Mild Traits**
Autism Spectrum	AQ-10 (10 items)	Social communication, attention switching, sensory & detail processing	≥ 6 pts	**Negative**
Generalized Anxiety	GAD-7 (7 items)	Autonomic arousal, motor tension & uncontrollable cognitive worry	≥ 10 pts	**Minimal**
Major Depression	PHQ-9 (9 items)	Anhedonia, dysphoric mood, sleep disruption & neurovegetative fatigue	≥ 10 pts	**Minimal**
Executive Function	BRIEF-A (14 items)	Behavioral inhibit, cognitive shift, working memory & plan/organize	≥ 12 pts	**Superior**
Trauma & Stress	PCL-5 (8 items)	Intrusive flashbacks, avoidance, hyper-vigilance & acute reactivity	≥ 14 pts	**Negative**

Clinical Psychiatric Synthesis: Across all 8 standardized screening instruments, symptom scores fall within non-clinical baseline thresholds. Mild elevations in OCD ordering and symmetry subscales correlate directly with the candidate's ultra-high conscientiousness (97.5%) and meticulous verification rigor rather than ego-dystonic pathology.

8. Patient Protocol: Daily Neurocognitive Optimization Schedule

A precision hourly routine engineered to maximize dopamine baseline, compensate for working memory limits, and sustain high fluid reasoning.

Time Window	Protocol Phase	Neurological Mechanism & Behavioral Prescription
07:00 — 08:00	**Circadian Cortisol Awakening**	10-15 mins direct outdoor natural lux light exposure + 500ml cold hydration with electrolytes. Zero phone/social feeds for first 45 mins to safeguard prefrontal dopamine receptors.
08:30 — 12:00	**Deep Work (Externalized RAM)**	Execute 50/10 Pomodoro blocks. To bypass working memory buffer decay (>5 items), maintain a structured physical or markdown scratchpad. Never hold unwritten state variables in active memory.
13:30 — 14:00	**Neuroplasticity Conditioning**	15 minutes of adaptive Dual N-Back drills + 5 minutes of visual DMTS memory expansion. Expands dorsolateral prefrontal cortex (dlPFC) updating efficiency and interference resistance.
14:00 — 17:30	**High-Ceiling Systems Architecture**	Channel 100% Logical and Spatial dominance into complex systems design, machine learning algorithms, and vulnerability auditing. Prioritize thorough verification over rushed reflex.
20:30 — 22:30	**Parasympathetic Neural Fading**	20 mins Theta (6Hz) binaural beats or Solfeggio 528Hz from the Focus Engine. Cease blue-light screens 60 mins before sleep to maximize slow-wave memory consolidation.

9. Practitioner Protocol: Clinical Impressions & Differential Guidelines

Clinical Focus	Practitioner Guidelines & Recommended Action
Discrepancy Decoupling	If fluid reasoning markedly outpaces processing speed, report the General Ability Index (GAI). In 2e/neurodivergent profiles, processing speed measures peripheral input latency rather than core intelligence.
Differential Diagnostics	Differentiate obsessive ordering from high conscientiousness. Inattention symptoms should be cross-verified against working memory capacity rather than assuming primary ADHD.
Confirmatory Batteries	For formal evaluation, administer computerized Continuous Performance Tests (TOVA / CPT-3), the clinical Yale-Brown Obsessive Compulsive Scale (Y-BOCS), and the structured SCID-5.
Targeted Interventions	Prescribe state externalization (kanban, structured intervals), Cognitive Behavioral Therapy (CBT/ERP) for perfectionism, and Heart Rate Variability (HRV) resonant breathing.

FORMAL CLINICAL & PSYCHOMETRIC CERTIFICATION:
This document certifies that the candidate has completed all 5 primary cognitive batteries (168 items/trials) and all 8 standardized clinical psychiatric screeners (106 items). All cognitive transformations adhere strictly to the Cattell-Horn-Carroll standard N(100, 15). The candidate achieves a **Full-Scale IQ of 143 (99.7th Percentile, Very Superior Range, 95% CI: 135–147, SEM: 3.1)** with non-clinical psychiatric screening baselines. *Clinical Notice: This platform provides standardized educational and cognitive profiling. In accordance with clinical standards, it is not a formal substitute for a proctored, multi-session clinical battery administered by a licensed neuropsychologist.*